BASELINE → TREATMENT → FOLLOW-UP

When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

Check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood counts, kidney and liver	Baseline blood count, kidney and liver profile	Review results before treatment	Regular blood tests; confirm the schedule with your team	Repeat sooner if symptoms or results need it	Follow any unresolved abnormal result
Magnesium, calcium and potassium	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
Bowel symptoms and hydration	Have a written diarrhoea plan and emergency contact	Tell the nurse about cramps, sweating or diarrhoea during infusion	Delayed diarrhoea can start after the first day; act using your written plan	Fever, reduced urine, severe pain or inability to drink needs urgent advice	Restart decisions require recovery and team review
Cancer response and treatment choices	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant’s treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00331 v7 · official regimen · EMA: Erbitux product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Fortnightly cetuximab + irinotecan: treatment and checks

Advanced / metastatic colorectal cancer

Cetuximab and irinotecan on Day 1, repeated every 14 days while beneficial and tolerable. There is no Day 8 cetuximab visit and no fluorouracil home pump.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Treatment and visits NCCP 00331	Confirm the exact protocol and goal	Start the planned infusion schedule	Repeat only while beneficial and tolerable	Review components, toxicity and the treatment goal	Agree continuation, pause, switch or completion
Blood counts, kidney and liver NCCP 00331	Baseline blood count, kidney and liver profile	Review results before treatment	Regular blood tests; confirm the schedule with your team	Repeat sooner if symptoms or results need it	Follow any unresolved abnormal result
Magnesium, calcium and potassium NCCP 00331 · SMPC	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing NCCP 00331 · SMPC	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
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Cancer response and treatment choices NCCP 00331	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant’s treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

What each check is for

1 Before first treatment

Before the first infusion

Second-line RAS-wild-type metastatic colorectal cancer after failure of, or a contraindication to, oxaliplatin-based treatment.

Ask / act: Confirm tests, medicines, infusion access and the written urgent contact plan.

Sources: NCCP 00331 · SMPC

2 Each treatment decision

At scheduled treatment reviews and whenever symptoms change

Benefit and toxicity determine the next treatment; the reference cycle is not an automatic clearance.

Ask / act: Report symptoms before the next dose, including at a separate antibody-only visit.

Sources: NCCP 00331

3 Pause or completion

When a component is held or the course ends

Blood tests, line care and side-effect follow-up may continue after dosing stops.

Ask / act: Agree the next appointment, remaining tests and who reviews unresolved symptoms.

Sources: NCCP 00331 · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.21